

Daily Content Intel

May 29, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, the American College of Sports Medicine just published their first update to the resistance training position stand in 17 years. They pulled 137 systematic reviews. Over 30,000 participants. The biggest evidence review on lifting that has ever been done.

The headline finding lands like a punch. Consistency beats perfection.

Specifically, training all major muscle groups at least twice a week matters more than the program design. Bands work. Bodyweight works. Machines work. Free weights work. That two-day-a-week, hit-everything floor is

what drives the gains.

The study also looked at what does not move the needle for the average adult. Training to failure? Did not move the needle. Complex periodization? Did not move the needle. Machines versus free weights? Did not move the needle. Chasing the perfect rep range? Did not move the needle.

Here's where I land as a PT. Most of the active adults I see in the clinic have read every podcast about volume landmarks, and they still can't find two days a week to lift. The problem is showing up.

If you're an athlete, the details matter, and I program them. If you're an active adult chasing strength and longevity, the boring answer wins. Pick two days. Hit everything. Repeat for a decade.

Be Good. Help Someone. Learn Lots.

Hook: The biggest update to resistance training guidelines in 17 years just landed.

Spoken script (word for word):

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range? Did not move the needle.

Here's where I land as a PT. Most of the active adults I see in the clinic have read every podcast about volume landmarks, and they still can't find two days a week to lift. The problem is showing up.

If you're an athlete, the details matter, and I program them. If you're an active adult chasing strength and longevity, the boring answer wins. Pick two days. Hit everything. Repeat for a decade.

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On-screen text seeds:

- 0:00 First ACSM RT update in 17 years
- 0:10 137 systematic reviews, 30,000+ participants
- 0:20 Headline: consistency beats perfection
- 0:30 2x/week, all major muscle groups
- 0:40 Bands. Bodyweight. Machines. Barbells. All work.
- 0:55 Failure, periodization, machines vs free weights: did not move the needle

- 1:10 Pick two days. Hit everything.
Repeat for a decade.

Caption (copy-paste ready):

The biggest resistance training guideline update in 17 years just dropped.

137 systematic reviews. Over 30,000 people. The verdict from the American College of Sports Medicine: consistency beats perfection.

Two days a week. All major muscle groups. Bands, bodyweight, machines, or barbells. That floor is what drives the gains.

Training to failure, complex periodization, machines versus free weights, the perfect rep range: none of it reliably changed outcomes for the average adult.

For the active adult chasing strength and longevity, the boring answer wins. Pick two days. Hit everything. Repeat for a decade.

For my athletes, the details still matter, and I program them.

Dr. Lars Stevenson, PT, DPT
threshold.clientsecure.me

#StrengthTraining #ResistanceTraining
#ACSM #PhysicalTherapy #Longevity
#StrengthAndConditioning #ActiveAdults
#Athletes #InjuryPrevention #LiftingForLife

Personalize this

- Which Threshold active-adult patient from the last year is the cleanest "consistency over complexity" story (showed up twice a week for 12 months and the transformation speaks for itself) — name them in voiceover and cut to a single before/after clip
- What is your actual 2-day full body template for a 50-year-old active adult with 45 minutes per session — drop the exact session into the on-screen text so the reel is prescriptive, not just philosophical
- What is the one line you say in onboarding when someone asks "which lift should I do first" — that flat answer is the close

Screenshots:

- title | <https://acsm.org/resistance-training-guidelines-update-2026/> | "ACSM Unveils Landmark 2026 Resistance Training Guidelines — First Update in 17 Years"
- physiology | <https://www.eurekalert.org/news-releases/1119943> | "The new recommendations, published by the American College of Sports Medicine (ACSM) as a Position Stand, are based on 137 systematic reviews involving more than 30,000 participants, making them the most comprehensive resistance-training guidelines to date."
- actionable | <https://www.eurekalert.org/news-releases/1119943> | "Training all major muscle groups at least twice a week matters far more than chasing the idea of a 'perfect' or complex training plan."

Sources:

- <https://acsm.org/resistance-training-guidelines-update-2026/>
- <https://www.eurekalert.org/news-releases/1119943>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, here's a fight I've been hearing inside physical therapy for my entire career. Manual therapy versus exercise for chronic low back pain. Which one wins?

A 2025 meta-analysis from the European Journal of Pain just answered it. Six randomized trials. 743 patients with chronic non-specific low back pain. Exercise versus manual therapy as a standalone treatment.

The result? No clinically relevant difference, right? Pain intensity was a tie. Disability favored exercise slightly at six months, but the size of the effect was too small to matter on its own.

So here's my hot take. The hands-on versus exercise turf war is a waste of clinical energy. Pick the lever that gets

the patient moving better, and stop selling the religion.

In my clinic, here's what I actually do. If a patient's locked up and afraid to move, I use my hands first to give them a window. Then I load the spine inside that window the same day. Manual therapy buys me the door. Loading's what keeps it open.

If your PT spends 45 minutes of an hour on a table doing soft tissue and 15 minutes on three exercises, the math is upside down. Hands open the door. Loading walks you through it.

Be Good. Help Someone. Learn Lots.

Hook: I'm a PT, and I'm telling you the manual therapy versus exercise war is over.

Spoken script (word for word):

Okay, here's a fight I've been hearing inside physical therapy for my entire career. Manual therapy versus exercise for chronic low back

pain. Which one wins?

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The result? No clinically relevant difference, right? Pain intensity was a tie. Disability favored exercise slightly at six months, but the size of the effect was too small to matter on its own.

So here's my hot take. The hands-on versus exercise turf war is a waste of clinical energy. Pick the lever that gets the patient moving better, and stop selling the religion.

In my clinic, here's what I actually do. If a patient's locked up and afraid to move, I use my hands first to give them a window. Then I load the spine inside that window the same day. Manual therapy buys me the door. Loading's what keeps it open.

If your PT spends 45 minutes of an hour on a table doing soft tissue and 15 minutes on three exercises, the math is upside down.

Hands open the door. Loading walks you through it.

Be Good. Help Someone. Learn Lots.

On-screen text seeds:

- 0:00 Manual therapy vs exercise: the PT culture war
- 0:08 2025 European Journal of Pain meta-analysis
- 0:14 6 RCTs, 743 patients, chronic low back pain
- 0:22 No clinically relevant difference as standalone
- 0:32 Pain: tie. Disability: slight edge to exercise, too small to matter.
- 0:50 In my clinic: hands open the door, loading walks through it
- 1:05 45 minutes on the table + 15 minutes of exercise = upside down
- 1:15 Pick the lever that gets the patient moving

Caption (copy-paste ready):

The manual therapy versus exercise war in physical therapy is over.

A 2025 European Journal of Pain meta-analysis. Six RCTs. 743 patients with chronic low back pain. As standalone treatments, no clinically relevant difference between hands-on and loading.

Pain intensity was a tie. Disability favored exercise slightly at six months, but the effect size was too small to matter on its own.

In my clinic, hands buy me the door. Loading is what keeps it open. If your PT spends 45 of 60 minutes on a table doing soft tissue and 15 minutes on three exercises, the math is upside down.

Pick the lever that gets the patient moving better. Stop selling the religion.

Dr. Lars Stevenson, PT, DPT It's time to cross your threshold.

threshold.clientsecure.me

#PhysicalTherapy #LowBackPain

#ManualTherapy #ExerciseTherapy

#ChronicPain #BackPain #PainScience #PT

#DPT #Rehab

Personalize this

- Which chronic LBP patient from the last year started with "my last PT just rubbed my back for an hour and a half" — name them so the data has a face on it
- What is your specific day-one protocol for a fearful, guarded LBP patient: which manual technique opens the door, and which loading exercise walks them through it the same visit (be specific, give the prescription)
- What is the one sentence you tell patients about why Threshold is not a manual-therapy-only practice — film that as the punchline so the hot take lands clinical instead of preachy

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12314856/> | "Exercise Therapy Versus Manual Therapy for the Management of Pain Intensity, Disability, and Physical Function in People With Chronic Low Back Pain: A Systematic Review With Meta■Analysis and Meta■Regression"

- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12314856/> | "There were statistically significant differences with a small effect in favour of ET for more than 6 months (SMD = -0.25 ; 95% CI $[-0.07, -0.43]$, $p = 0.007$)"
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12314856/> | "Although both therapies alone might produce small beneficial effects on different clinical parameters, the choice between ET and MT should be reconsidered in favour of CPPP, considering individual patient factors, preferences, and the clinical context."

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC12314856/>
- <https://pubmed.ncbi.nlm.nih.gov/40747709/>

Reference

Runner-up topics

- 2026 JFMK systematic review on collagen supplementation for tendon: inconsistent results across trials, good myth-busting hot take on "collagen for joints"
- 2026 Nutrition Reviews systematic review: creatine + cognition in older adults, muscle-brain axis angle for active-adult patients
- Sports Med 2026 scoping review: strength training as injury prevention in team sports, coach-facing reel

Sources scanned today

- Newsletter digest (2026-05-29): three sources, all promo (Mercola headlines x2, Mike T. Nelson summit ad). Nothing citable.
- PubMed: most fresh PT RCTs were stroke telerehab and Parkinson mobility, off-audience.
- ACSM: 2026 Position Stand on Resistance Training, the first update in 17

years (strong hit).

- EurekaAlert / ACSM press release: same Position Stand, with prescription specifics.
- European Journal of Pain (2025) meta-analysis: exercise vs manual therapy as standalone for chronic LBP, 6 RCTs, 743 patients (strong hit, on-audience hot take).
- Sports Med 2026 scoping review: strength training for injury prevention and performance in team sports.
- Nutrition Reviews 2026: creatine + cognition in older adults systematic review.
- JFMK 2026 systematic review: collagen supplementation on tendon outcomes (mixed results).
- Examine, Peter Attia, Huberman, Iron Culture: no fresh standout this week.
- Reddit r/AdvancedFitness, r/physicaltherapy: nothing above the noise floor.